



Potentially Harmful Items Management for Mental Health Patients SOP

Scope

Site	Department, division, or operational area	Applicable to
Royal Perth Bentley Group (RDBG)	Mental Health Division (MHD) Inpatient wards/services	All staff

Purpose

To identify and mitigate patient risks to self, staff and others by minimising or preventing harm from potentially unsafe personal items.

Background

Mental health (MH) patients are often admitted because there is an underlying risk to self or others. Self-harm and harm to others for a patient can come in many forms including hanging, cutting, ingesting substances, burning or aggression. Commonly, patients who die by suicide on MH wards do so by hanging or strangulation in private areas such as bedrooms and bathrooms.^{1,2}

Links within document

- [Appendix I: Prohibited or restricted items list for secure wards - EmyU, Ward 6, MHU and Ward 7](#)
- [Appendix II: Prohibited or restricted items list for MHEC and Ward 10](#)
- [Appendix III: Prohibited or Restricted Items list for open wards - TCU, Ward 2K and Ward 8](#)
- [Appendix IV: STEP 1 - Checking patient belongings/property in an inpatient setting](#)
- [Appendix V: STEP 2 - Searching patients in inpatient wards](#)
- [Appendix VI: Management of seized items, substances, weapons, or uncooperative patients \(voluntary/involuntary patients\)](#)

Aims and approach

- Environmental safeguards, i.e., elimination or reduction of environmental hazards or unsafe items are identified as effective in preventing patient harm or suicide
- Therapeutic interventions used in conjunction with other interventions, e.g., therapeutic rapport, adequate risk assessment, treatment of underlying conditions, optimising patient visibility and supervision, consideration of issues of privacy, dignity, and least restrictive practice, coordinated teamwork and communication, etc.²

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Version: 3.1

Endorsed by: Clinical Practice Committee 07/12/2023

Authorised by: No Patient Harm Committee 23/01/2024

DMR Amendment: Clinical Practice Committee Chair OOS 26/02/2025

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Definitions

Unsafe Items	Any object that may potentially pose a risk to a patient's safety, other patients, or staff, including adapted everyday items. Examples include sharp instruments, glassware, pins, hair ties, inhalants, belts, shoelaces, or clothing. Additional items deemed prohibited or restricted within specific wards or units - refer to Appendices I , II or III .
Firearms	A lethal barrelled weapon of any description, including replica firearms or imitation. See Firearms Act 1996 and refer to EMHS Alcohol and Prohibited Substances Policy .
Controlled and Prohibited Weapons	Items defined under the WA Weapons Regulations 1999 - i.e. scissors, syringes, furniture, (see Schedule 1 - Prohibited Weapons and Schedule 2 - Controlled Weapons and refer to EMHS Alcohol and Prohibited Substances Policy).
Weapons	Lawful items defined under the WA Weapons Act 1999 and Weapons Regulations 1999 , e.g. sports related activities, speargun and cattle prods.
Prohibited drug/ other substances, alcohol	A prohibited drug as defined in the Misuse of Drugs Act 1981 section 3(1) and/or other substances or alcohol as defined in the EMHS Alcohol and Prohibited Substances Policy .
Checking	The process of staff asking a patient or visitor or personal support person (PSP) or Guardian to declare belongings to ensure assessment of items that may pose a serious risk to the health and safety of the patient or others on admission or return to a MH inpatient area.
Searching	Refer to EMHS Searching of Mental Health Patients Belongings and Property Policy .

Procedure details

Checking and searching

On admission, return from leave and when clinically indicated, nursing staff, or treating team are to:

- Explain property and search processes to patients, PSP, or Guardian, including the rationale for the clinical decision to support removal or storage of items or restrict access to means (e.g., individual risk, environmental hazards) and document in the integrated notes
- Discuss and provide the patient with the [Property Checks and Safety Searches Consumer Leaflet](#)
- Utilise *Mental Health Patient Property Check / Change in Access Template in the DMR*

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Procedure details – Checking and searching cont'd

Perform **STEP 1: Check patient belongings or property** (see flowchart in [Appendix IV](#)).

- For **all** patients on admission, clinicians are to check bags, ask patient to remove outer layer of clothing and turn out pockets. Document the outcome of the check in the *Comprehensive Care Plan*, patient property in the property form and in the patient healthcare record. Note any further details relating to restricting access where applicable

Note:

- Patient risk assessment must be completed prior to allowing medical devices** (e.g., continuous positive airway pressure [CPAP] machine) on wards or units
 - NO plastic bags (except Transitional Care Unit [TCU]), lighters, matches, vape products and e-cigarettes are not permitted in all wards or services**
- When clinically indicated **and returning from each episode of leave, check patient belongings or property and document in the *Mental Health Patient Property Check / Change in Access Template in the DMR*(except TCU) Complete metal detector searches** to assist with checking patient's belongings and property in all authorised units and some, selected unauthorised units. **Document in the patient's healthcare record any further details relating to restricting access where applicable**
- Any item can be deemed restricted due to risk of harm to patient or harm to others in all services and requires a clinical decision
 - Refer to additional examples of prohibited or restricted items and items not appropriate as per specific units in Appendices [I](#), [II](#) and [III](#): Prohibited or restricted items lists
- As per **Visitor Management Policy**, either check all visitor belongings or request the visitor to store belongings prior to entrance to the ward or unit. Document in the property book, as relevant
- Store patient property in secure storage or give to PSP, Guardian, or visitor, if appropriate
- Complete or update patient's *Risk Assessment Management Plan (RAMP)* on Psychiatric Services On-Line Information System (PSOLIS) or *Progressive Risk Assessment (PRA)*, including the risk to patient in relation to potentially unsafe items
 - Escalate safety or risk concerns to Nurse Unit Manager (NUM), Shift Coordinator, MH Clinical Nurse Specialist (CNS), Medical Officer (MO), Duty Medical Officer (DMO) or Psychiatric Registrar and discuss and document agreed management strategy in the integrated notes, care plan and *Deliver What Matters Most (WMM) Form*.

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Procedure details – Checking and searching cont'd

Perform STEP 2: Search patient if concerns arise or where clinically indicated

- Consider the below indicators for a search:
 - A patient acting in a suspicious way, i.e. hiding or concealing an item that might compromise the health and safety of themselves or another person
 - Where a patient or other(s) may be at potential risk of imminent harm
 - If found prohibited items or suspect a patient may be carrying prohibited drugs or other substances, weapons or firearms etc. as per the **EMHS Alcohol and Prohibited Substances Policy**
 - Previous history of the patient including aggression, violence, self-harm, etc.
 - Alerts on PSOLIS or WebPSOLIS and document outcomes in the DMR
 - Complete search as per [Appendix V: STEP 2 Searching patients in inpatient wards](#).

Uncooperative patients (voluntary or involuntary patients)

- If the patient does not agree to removal of items during treatment, immediately commence close visual observations and inform the Shift Coordinator, MH CNS, NUM, and MO or DMO, as this behaviour may result in a different management plan and clinical area being required.

Returning stored items

The Allocated Clinician or Treating Team is to:

- Complete a risk assessment and use clinical judgement to determine whether to return e.g. unsafe items to the patient, PSP or guardian on discharge and document the outcome of decisions in the patient's healthcare record
 - Prohibited substances and weapons (prohibited or controlled) or firearms (as per the **EMHS Alcohol and Prohibited Substances Policy** and **EMHS Security Policy**) are not returned
 - Consider that alcohol (unopened vessels) or other substances or items not prohibited by law may be returned if it is safe to do so for patients and/or others. Follow guidance as per the **Alcohol and Prohibited Substances Policy**
 - Complete other documentation as per **Patient Property Policy**.

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Procedure details cont'd

Patient assessment and observation

Clinicians are to escalate to the Shift Coordinator and/or alert security if they have any concerns about their personal safety through this process, as required.

Where risk is identified or there are safety concerns relating to use or possession of potentially unsafe items, prohibited substances and weapons or additional, prohibited or restricted items within specific wards or units (refer to Appendices [I](#), [II](#) and [III](#)) on admission and during treatment, the Clinician or Treating Team is to:

- Provide higher levels of visual observations (e.g. use room with good proximity to the nurse station) and follow escalation guidance as per *RAMP* or *PRA*. Refer to **Special and Supportive Observations Policy** and/or **Progressive Risk Assessment SOP**
- Actively communicate risk to the treating team, e.g. changes in mental state, if the patient has a history of carrying or using weapons while in a health service or restrictions to items and give prominence in MDT review, handovers, huddles and/or team discussions
- Raise an alert on PSOLIS or WebPSOLIS, on iClinical Manager, electronic handover, and document in the patient's healthcare record
- If a staff member finds or suspects a patient may be in possession or use of a prohibited drug or other substance, weapon, firearms, or medication that is not their own or not prescribed by the treating team, refer to [Appendix VI: Management of seized items, substances, weapons, or uncooperative patients \(voluntary/involuntary patients\)](#)
- Where indicated, NUM, CNS or Shift Coordinator request medical staff to attend the clinical area at the earliest possible time to review the patient's treatment plan
- For management and escalation of care, refer to:
 - **Prevention and Management of Aggressive Behaviours within Mental Health Inpatient Setting SOP**
 - [Emergency Procedures Manual](#) (relevant to local site, e.g. RPH, BHS, TCU)
 - **Recognising and Responding to Acute Deterioration (RRAD) Policy**
 - **RRAD in Mental State, Cognition and Behaviour SOP**
 - **Clinical Care of People Who May Be Suicidal Policy.**

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Procedure details cont'd

Ward or service environment

At admission or at any point during treatment the designated Nursing staff or Treating Team are to complete:

- **Ward, courtyard supervision or other environmental management strategies** to mitigate adverse effects of use of potentially unsafe items or environmental hazards as appropriate based on risk assessment, including:
 - Reducing barricade options in the room e.g. implement agreed processes for accessing areas that can be locked such as ensuites
 - Ward specific restrictions e.g. recommended specific cutlery and moderated patient access to courtyards.
- Report any incidents on Datix Clinical Incident Management System (CIMS) or Combined Hazard or Incident Report (CHoIR) and/or Notifiable Incident to the Chief Psychiatrist (CP), if required.

Observation regarding kitchen areas and Activities of Daily Living (ADLs)

To promote risk mitigation and prevention of potentially harmful items or environmental hazards, Clinicians are to:

- Ensure at least one member of nursing staff is present in the dining area during mealtimes
- Advise Patient Support Services (PSS) of any patients at risk of clinical or behavioural concerns and provide support in delivery of meals as required
- Ensure appropriate supervision of the patient, including formal assessment of ADLs for inpatient areas that provide ADLs, kitchen activities or courtyard activities that may include potentially harmful items (i.e. sharp knives, equipment) as required. **Patients must be supervised during use of these items** and sharps locked in the drawer provided between uses where required
- Prior to commencement of an ADLs or kitchen activity and post activity/ADL, reconcile and register all equipment or items on an equipment checklist and ensure the equipment register items or areas are locked when not in use
 - If an item is missing or not returned in the reconciliation post activity or ADL, complete immediate search of the area
 - Escalate to the NUM or Ward Shift Coordinator then CNS or Senior RN, MO or DMO if item not found
 - Review or complete patient *RAMP* or *PRA*, patient care management plan and consider increasing visual observations for patients deemed at risk of possessing or using a potentially harmful item.
- Ensure no additional equipment is brought into ADLs or kitchen areas
- Check delivered meals, e.g. Uber Eats, cutlery, other delivered items, and apply restrictions or risk management strategies based on risk assessment, particularly for patients at risk of self-harm, harm to others and/or harm from others
- NUM, CNS, or Occupational Therapist is to complete regular quarterly review of equipment lists to monitor missing items and the integrity or condition of items
- To support safety of patients using laundry facilities, refer to the **Laundry Facilities Use by Mental Health Consumers SOP**.

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Compliance monitoring

- Datix CIMS, CHoIR or Notifiable incidents to Chief Psychiatrist
- NUMs or CNSs responsible for:
 - quarterly review of equipment lists
 - inpatient Work Health Safety Committee and relevant safety and quality meetings
- Compliments or Complaints Management System.

Facilitator

Marika Spaseska, Policy Officer, Innovation and Education, RPBG

Authors / acknowledgements

We acknowledge the following previous site endorsed work and/or contributors used to compile this document.

Jill Spears, Coordinator of MH Nursing, MHD, RPBG 20/04/2021

Review authors

Dr Calum Smith, A/Director of Clinical Training, Consultant Psychiatrist, MHD, RPBG

Cherry Acabo, Ward 6 NUM, MHD, RPBG

Jennifer Hall, CNS, TCU, RPBG

Hannah Midani, Physical Health Care Clinical Nurse Specialist (CNS), MHD, RPBG

Fenece Collett, Coordinator of Nursing MH, BHS, MHD, RPBG

Sarah MacKinnon, A/Coordinator of Nursing MH, RPH, MHD, RPBG

Shannon Wilson, NUM, Ward 7, MHD, RPBG

Sarah Galbraith, NUM, EMyU, MHD, RPBG

Matthew Murcia, NUM, MHU, MHD, RPBG

Shan Santos, NUM OA, Ward 10, MHD, RPBG

Manoh Dumbuya, NUM, Ward 8, MHD, RPBG

Kirstie Kemble, Mental health Divisional Safety & Quality Coordinator, RPBG

Dr Joanne Chua, Consultant Psychiatrist, MHD, RPBG

Dr Vinesh Gupta, Medical Co-Director, MHD, RPBG

Feedback received

Caylee Lange, A/Senior Project Officer, Comprehensive Care, RPBG

Lyn Grattidge, CNS, IPM, RPBG

Luke Roscoe, Security Supervisor, Royal Perth Hospital, EMHS

Jeremy Coulson, Area Security Manager, EMHS

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Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Potentially Harmful Items Management for Mental Health Inpatients SOP through consultation with the RPBG Aboriginal Workforce Engagement Group which includes all Aboriginal staff (clinical and non-clinical) across EMHS including the Aboriginal Health Liaison Officers, Aboriginal Health Strategy Team, and Aboriginal Acute Care Coordination Team to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 3016).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

Related policies, practice standards, clinical guidelines

RPBG Policy Hub

- [Ligature Risks Management, Safe Use and Storage of Ligature Cutters Policy](#)
- [Alcohol and Drug Intoxication and Withdrawal Management Clinical Guideline](#)
- [Comprehensive Care SOP](#)
- [Mental Health Patient, Carer and Visitor Support Post-Critical Incident SOP](#)
- [Progressive Risk Assessment SOP](#)
- [CCTV Monitoring in Mental Health Inpatient Areas SOP](#)
- [Clinical Handover Policy](#)
- [Minimisation and Safe Management of Restraint and Seclusion \(for Mental Health Areas\) CPS](#)
- [Visitor Management Policy](#)
- [Patient Property Management Policy](#)
- [Special and Supportive Observations Policy](#)
- [Recognising and Responding to Acute Deterioration \(RRAD\) Policy](#)
- [RRAD in Mental State, Cognition and Behaviour SOP](#)
- [Clinical Care of People Who May Be Suicidal Policy](#)
- [Laundry Facilities Use by Mental Health Consumers SOP](#)

EMHS Policy Hub

- [Notifications Requirements Under the WA Mental Health Act 2014 Policy](#)
- [Security Policy](#)
- [Security Services Manual](#)
- [Alcohol and Prohibited Substances Policy](#)
- [Searching of Mental Health Patients Belongings and Property Policy](#)

Department of Health Policy Hub

- [Statewide Standardised Clinical Documentation \(SSCD\) Policy](#)

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Related policies, practice standards, clinical guidelines cont'd

Other

- [RPBG Property Checks and Safety Searches Consumer Leaflet](#)
- [Mental Health Act 2014](#)
- [Work Health and Safety Act 2020](#)
- [Misuse of Drugs Act 1981](#)
- [Weapons Regulations – Weapons Act 1999](#)
- [Chief Psychiatrists Standards of Clinical Care](#)
- [Charter of Mental Health Care Principles](#)

Related forms

- *Comprehensive Care Plan*

Related national standards

ACSQHC NSQHS Standards 2nd Edition (2021): Standard 1: Clinical Governance; Standard 6: Communicating for Safety; Standard 8: Recognising and Responding to Acute Deterioration

NSMHS Standards (2010): Standard 2: Safety; Standard 3: Partnering with Patients; Standard 10: Delivery of care

References

1. Zero Suicide Institute. Zero Suicide Framework for Safe Suicide Care
2. Hunt, I.M., Windfuhr, K., Shaw, J., Appleby, L., Kapur, N. (2012). *National Confidential Inquiry into Suicide and Homicide. Ligature points and ligature types used by psychiatric inpatients who die by hanging: a national study*. Crisis. 2012 Jan 1, 33(2): 87-94. Doi: 10.1027/0227-5910/a000117.
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Appendix I: Prohibited or restricted items list for secure wards - EmyU, Ward 6, MHU and Ward 7

 Prohibited Items must be removed, stored, and returned on discharge	• Weapons of any type • Sharps, tools and metal objects • Needles, safety pins, blades • Steel capped boots, high heels • Coat hangers • Floral arrangements containing wires • Scissors • Ropes of any kind • Belts, lanyards, bandages • String, wool, or dental floss • Plugs, cords, cables or chargers with dimensions longer than 20cm and able to be tied in a knot • Shoelaces • Plastic bags	• Cigarettes, E-cigarettes, tobacco, lighters or matches • Glass or ceramic items • Keys • Ornaments • Metal cans • Clothing with cords of dimensions longer than 20cm and able to be tied in a knot • Jewellery with dimensions longer than 20cm and able to be tied in a knot • Scarves, ties, or tights • Handbags	• Sexual products • Alcohol and illicit substances • Any drug paraphernalia • Patient's own medications • Unidentified liquids or powders • All hazardous chemicals i.e. toxic or flammable substances • Hot water bottles • CDs or DVDs • Electrical items (e.g. speakers, headphones) with cords of dimensions longer than 20cm and able to be tied in a knot • Batteries
 Restricted Items must be used under supervision and returned to staff after use based on Risk Assessment	• Mouthwash • Nail polish/remover • Aerosol spray • Cotton buds • Permanent markers • Hair dryer/straightener/curler • Hair dye	• Electric and disposable razors • Art/craft supplies • Pencil sharpeners • Paperclips, glue, staples • Crutches, canes, walkers, wheelchairs • Financial documents, key cards • Nail clippers/tweezers	• Jewellery (except as above in Prohibited items list) • Electrical items • Energy drinks • Any herbal materials/essential oils • Headphones - wireless • Musical instruments
 Restricted Items kept in property room/secure storage and must be returned to staff	Store away RED flagged items Risk assess and supervise AMBER flagged items Items deemed safe and appropriate in GREEN can be retained by the patient When in doubt check with Shift Coordinator, NUM, or CNS before giving item to patient The aim is to provide a safe environment for patient, staff and PSPs, guardians, visitors while practising least restrictive care		

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Appendix II: Prohibited or restricted items list for MHEC and Ward 10

	Remove and store items. To be returned to patient on discharge. • Weapons of any type • Sharps, tools, and metal objects • Knitting needles, safety pins, blades, razors • Steel-capped boots, high heels • Coat hangers • Floral arrangements containing wires • Scissors	• Cigarettes, E-cigarettes, tobacco, lighters or matches • Glass and ceramic items • Media promoting violence • Metal cans • Ornaments • Hot water bottles • Ceramic mugs, cups, plates (MHEC only) • Patient's own medications • Bags with non-detachable straps	• Sexual products • Alcohol and illicit substances • Any drug paraphernalia • Unidentified liquids or powders • All hazardous chemicals i.e., toxic or flammable substances • Ropes of any kind • Plastic bags
	• Mouthwash • Nail polish/remover • Aerosol spray • Hair dye • Hair straightener, curler or dryer • Musical instruments • Straps which can be detached from bags, e.g., handbag straps • Plugs, cords (including corded electrical equipment), chargers with dimensions longer than 20cm and able to be tied in a knot • Energy drinks	• Clothing with cords of dimensions longer than 20cm and able to be tied in a knot • Belts, shoelaces, lanyards, and bandages • String, wool, or dental floss • Headphones (wireless) • Scarves, ties, tights • Jewellery • CDs and DVDs • Any herbal materials/essential oils • Ceramic mugs, cups, plates (Ward 10 only)	Note: Items in this category can be retained by patient with reduced supervision according to risk profile. If in doubt consult Shift Coordinator or CNS. Any changes to patient risk profile will require a reassessment of items retained by patient and removal of item(s) if a risk is identified.
	• Any items assessed as low risk, safe and appropriate can be retained by patients. • These items do not require staff supervision unless specific individual risk factors have been identified. • When in doubt check with Shift Coordinator, NUM, or CNS before giving items of potential risk to patients. • RPBG aims to provide a safe, therapeutic environment and minimise risk using the least restrictive practice.		

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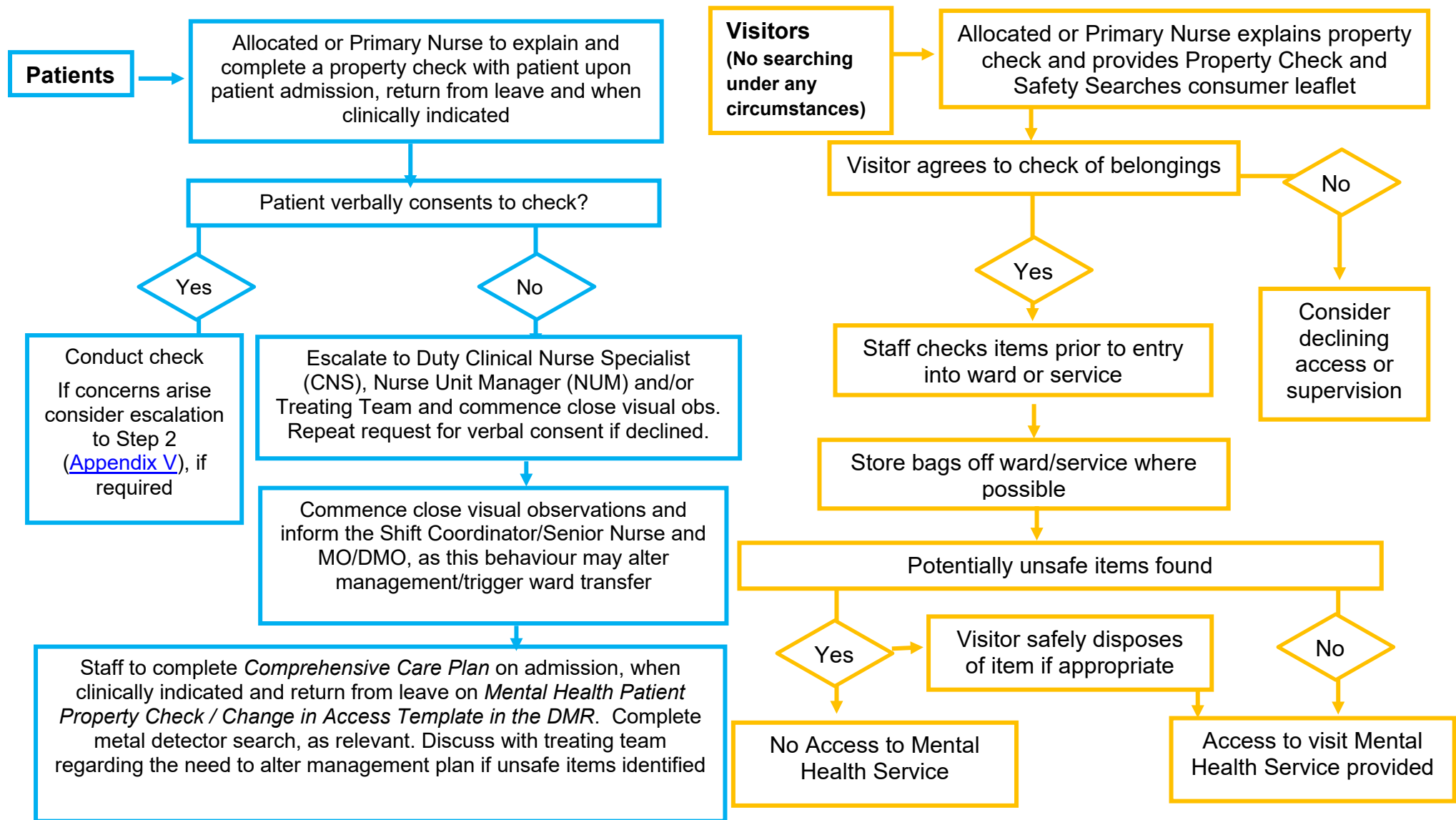
Appendix III: Prohibited or restricted items list for open wards – TCU, Ward 2K and Ward 8

 PROHIBITED items	Remove and store items. To be returned to patient on discharge. • Weapons of any type • Sharps, tools, and metal objects • Needles, safety pins, blades, razors • Steel-capped boots, high heels • Coat hangers • Floral arrangements containing wires • Scissors • Cigarettes, E-cigarettes, tobacco, lighters or matches	• Plugs, cords, chargers with dimensions longer than 20cm and able to be tied in a knot • Ropes of any kind • Glass or ceramic items • Metal cans • Ornaments • Hot water bottles • Ceramic mugs, cups, plates • Patient's own medications	• Sexual products • Energy drinks • Alcohol and illicit substances • Any drug paraphernalia • Any herbal materials/essential oils • Unidentified liquids/powders • All hazardous chemicals i.e., toxic or flammable substances • Plastic bags (Ward 2K & Ward 8 only)
 Restricted Items requiring SUPERVISION	Risk assess prior to use, most items require supervision. To be returned to staff after use. • Mouthwash • Nail polish/remover • Aerosol spray • Hair dye • Hair dryer/straightener/curler • Musical instruments	• Clothing with cords with dimensions longer than 20cm and able to be tied in a knot • Belts, shoelaces, lanyards, and bandages • String, wool, or dental floss • Headphones (wireless) • Scarves, ties, tights • Jewellery • CDs or DVDs	• Certain electrical items may be used (i.e. laptop without power cord) • Note: some patients may be allowed to keep certain items from this list without supervision.
 Low Risk Items allowed to be retained by patients	• Only items assessed as low risk, safe and appropriate can be retained by patients. • These items do not require staff supervision unless specific individual risk factors have been identified. • When in doubt check with Shift Coordinator, NUM, or CNS before giving items of potential risk to patients. • We aim to provide a safe, therapeutic environment and minimise risk using the least restrictive practice.		

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Appendix IV: STEP 1 - Checking patient belongings/property in an inpatient setting



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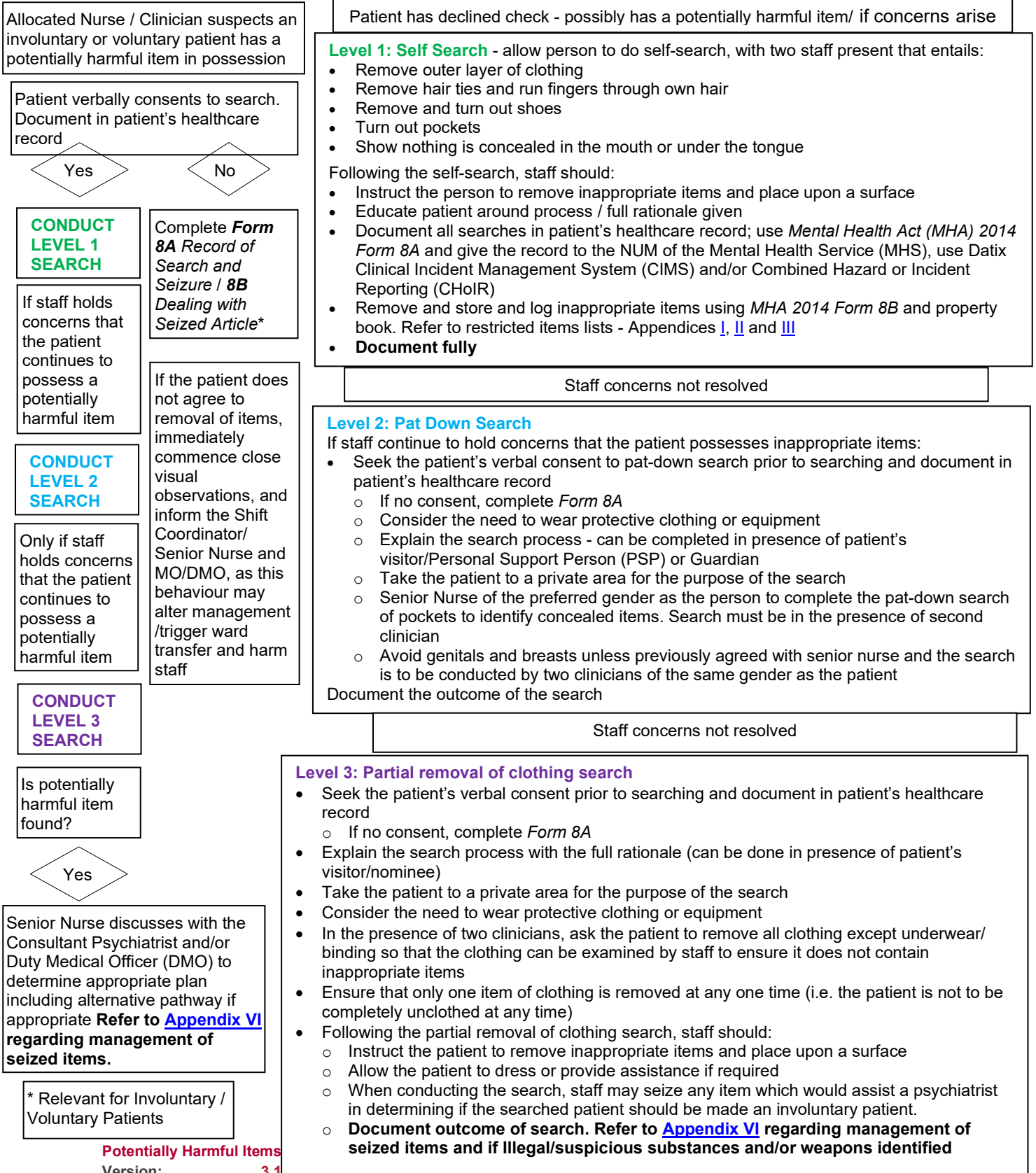
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Appendix V: STEP 2 - Searching patients in inpatient wards

Inpatient Ward: Searching patients

Note: The aim of this process is to mitigate risks of patient access to potentially harmful items.



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Appendix VI: Management of seized items, substances, weapons, or uncooperative patients (voluntary/involuntary patients)

- If the patient does not agree to removal of items, Clinicians are to immediately commence close visual observations and inform the Shift Coordinator, NUM/ MH CNS and MO/DMO, as this behaviour may result in a different management plan and clinical area being required.

Isolate/secure items

Clinicians are to:

- Assess if there is immediate danger. If weapons are found / in emergency situations, activate appropriate Emergency Procedures as required (Code Black, Code Purple, 000)
- If it is safe to do so, confiscate prohibited items such as alcohol, illicit drugs, and weapons* as defined in the **EMHS Alcohol and Prohibited Substances Policy** until Security arrive
***Never** handle/take possession of a firearm, a controlled and/or prohibited weapon from a patient, PSP, guardian and/or visitor who is holding it, even if the person is willing to hand it over. Pick up the item only after it is first placed on a surface. Refer to **EMHS Alcohol and Prohibited Substances Policy**
- Advise the Shift Coordinator, NUM, CNS
- The Shift Coordinator, NUM, CNS, After Hours CNS and treating doctor will determine if further action is required, e.g. contact Security
- Call Security for assistance regarding identification of and/or safe removal/handling of prohibited items/weapons or firearms, other suspicious items, etc., when necessary. Refer to **EMHS Alcohol and Prohibited Substances Policy** and **EMHS Security Policy**
- Liaise with the patient, PSP, guardian and/or visitor to ensure consent to retrieve and secure items or other substances not prohibited by law and pose no imminent risk or danger to patient and/or staff or others. Store item(s) in a secure locker or safe situated out of the clinical area or give to the patient's Guardian or PSP, or call Security for storage and/or disposal, as appropriate
- Store valuables and cash in the cashier's office or alternative, safe storage.

Illegal substances and/or weapons or firearms identified:

- If the patient does not acknowledge possession and refuses to cooperate, then the matter should be referred to the Police
- Security Officers do not have the right to search a person or their personal belongings without that person's permission*

*Staff of a mental health service may request another person (e.g. security staff) to assist in carrying out the search. Both the mental health service staff and the security staff assisting may lawfully use reasonable force in conducting the search. (Refer to **EMHS Security Policy** and Section 163 of the **MHA 2014**)

Potentially Harmful Items Management for Mental Health Inpatients SOP

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